

Classify by anatomy and damage

Angle closure can be asymptomatic; gonioscopy and evidence of damage determine the category.

Primary angle-closure spectrum

Category	Distinguishing finding
PACS	Angle contact; no PAS, raised pressure or glaucomatous damage
PAC	Angle contact with PAS and/or raised pressure; no glaucomatous damage
PACG	Angle contact with glaucomatous optic neuropathy
Acute presentation	A clinical attack; not required for the chronic categories

Use screening appropriately

Van Herick assessment estimates peripheral chamber depth but remains a screening method. Suspected narrowing needs gonioscopy to assess iridotrabecular contact and synechiae. Do not use symptom absence or a normal acuity result to exclude an occludable angle. The distinction between PAC and PACG is optic nerve damage, not merely a high pressure reading. Secondary causes must be considered before applying a primary classification.

Iridotomy does not solve every mechanism

A patent opening relieves relative pupillary block but may leave other contributors to angle narrowing.

After angle assessment

What mechanism is dominant?

Relative pupillary block

Iridotomy equalises chamber pressure and reduces iris bowing.

Lens enlargement or displacement

A cataractous intumescent or subluxed lens can contribute to secondary closure.

Non-pupillary or synechial obstruction

Persistent angle compromise may require further intervention despite iridotomy.

Reassess instead of assuming success

Check the angle and pressure after treatment. In primary angle closure or PACG, inadequate widening or persistent pressure elevation lowers the threshold for additional care. Pharmacological pupil dilation can worsen block in a predisposed eye; this is an anatomical caution, rather than a blanket prohibition for every glaucoma patient. The fellow eye may share the predisposition even when it has never been painful.